Síndrome do ovário policístico

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Sinais e sintomas

- Anovulação crônica, não continua
- Excesso de androgênios, oligoamenorreia ou amenorreia, hirsutismo, acne e alopecia.
- Pelos faciais e corporais excessivos
- Associação com resistencia insulinica e sindrome metabólica.
- Disturbios metabólicos do carboidrato – 45 %, ganho de peso.
- Abortamento espontâneos, diabetes gestacional, dislipidemia e HAS.

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MULHERES COM SOP APRESENTAM RISCO AUMENTADO DE:

Sobrepeso e Obesidade
Pré-diabetes e diabetes mellitus tipo 2
Dislipidemia e doenças cardiovasculares
Apneia obstrutiva do sono
Doença hepática gordurosa não-alcoólica
Distúrbios de humor, ansiedade, depressão
Câncer de ovário e endométrio
Abortamento espontâneo
Complicações obstétricas (diabetes mellitus gestacional, doença hipertensiva específica da gravidez, pré-eclâmpsia e partos prematuros)
Vontade exagerada por doces/carboidratos,
Cansaço/fadiga
Dificuldade em engravidar
Dificuldade em perder peso
Tireoidite de Hashimoto.

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Clinical Trial > Horm Res. 2009;71(3):148-54. doi: 10.1159/000197871. Epub 2009 Feb 3.

Determinants of interleukin-6 and C-reactive protein vary in polycystic ovary syndrome, as do effects of short- and long-term metformin therapy

Tasoula Tsilchorozidou [1], Vidya Mohamed-Ali, Gerard S Conway

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Contents lists available at ScienceDirect

Taiwanese Journal of Obstetrics & Gynecology

journal homepage: www.tjog-online.com

Original Article

Serum ferritin levels and polycystic ovary syndrome in obese and nonobese women

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A B S T R A C T

Objective: The aim of this study is to evaluate serum ferritin levels and polycystic ovary syndrome (PCOS)-related complications in obese and nonobese women.
Materials and methods: This retrospective study included 539 (286 with PCOS and 253 without PCOS).
Results: Serum ferritin correlated with menstrual cycle length, sex hormone-binding globulin, total testosterone, androstenedione, triglyceride, and total cholesterol in both obese and nonobese women. Obese women with high ferritin levels exhibited higher insulin resistance, impaired glucose tolerance, and liver enzymes (glutamic oxaloacetic transaminase, glutamic pyruvic transaminase) than obese women with low ferritin levels. However, among nonobese women, insulin resistance and risk of diabetes were not significantly different between the high and low ferritin groups. Independent of obesity, hypertriglyceridemia was the major metabolic disturbance observed in women with elevated serum ferritin levels.
Conclusion: Elevated serum ferritin levels are associated with increased insulin resistance and risk of diabetes in obese women but not in nonobese women. However, higher serum ferritin levels were correlated with a greater risk of hyperglyceridemia in both obese and nonobese women. Therefore, hypertriglyceridemia in women with PCOS might be associated with iron metabolism.
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Diagnóstico

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NIH	Rotterdam	Sociedade AE-PCOS
Presença de dois critérios	Presença de dois dos três critérios	Presença de dois critérios
Disfunção menstrual Hiperandrogenemia e/ou hiperandrogenismo	Anovulação crônica Hiperandrogenemia e/ou hiperandrogenismo Ovários policísticos	Hiperandrogenemia e/ou hiperandrogenismo Disfunção menstrual e/ou ovários policísticos

Para todos os critérios assume-se que foram descartadas outras causas de hiperandrogenismo

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1. Oligomenorréia e/ou anovulação
2. Sinais clínicos e/ou bioquímicos de hiperandrogenismo: hirsutismo, acne, pele oleosa, queda de cabelo e/ou elevação de pelo menos um androgênio (testosterona total, androstenediona, sulfato de desidroepiandrosterona sérica (SDHEA).
3. Ovários policísticos (12 ou mais cistos com 2 a 9mm) ou volume ovariano >/= 10 cm3 .

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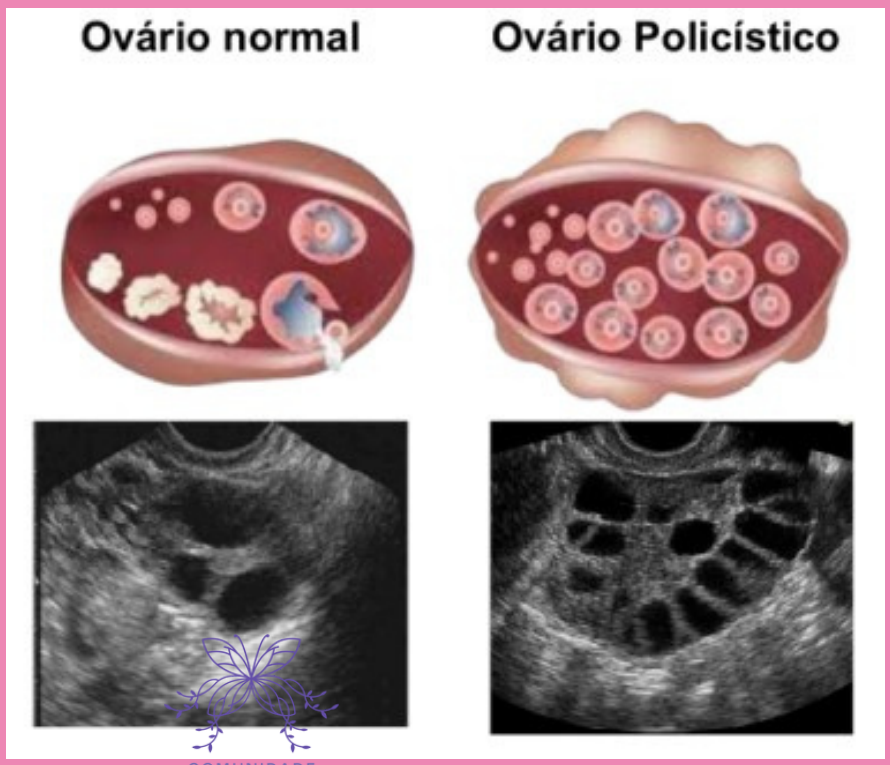

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Fenótipo	A	B	C	D
Oligo ou amenorreia	+	+	-	+
Hiperandrogenismo clínico e/ou laboratorial	+	+	+	-
Ovários policísticos à ultrassonografia	+	-	+	+

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Características	Fatores interferentes	Estratégias
Resistência à insulina	Excesso de açúcar, farinha refinada Acúmulo de gordura Excesso de toxinas	Low carb (menor que 100g/dia) ou Cetogenica (menor que 60g/50g/dia) Jejum intermitente Suplementos: berberina, ac R alfa lipoico, magnésio, zinco, coenzima Q10, vitamina D
Após uso de pílula (por suprimir ovulação)	Altos níveis de LH e prolactina de normal a alta	Ervas: Vitex (apenas depois de 4-6 meses que parou a pílula)
Inflamatória (impede ovulação)	Estresse Toxinas ambientais Permeabilidade intestinal Baixos níveis de vitamina D Altos níveis de PCR Alteração da tireoide	Proteção de barreira intestinal: glutamina, glicina NAC Vitamina D, Magnésio, zinco OBS: pode demorar entre 6-9 meses para regular
Outros fatores	Excesso de estrógeno, doença tireoidiana, dieta com excesso ou falta de carboidrato...	Analisar cada caso individualmente

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Vários fatores têm sido implicados no desenvolvimento da SOP, havendo componentes genéticos envolvidos, fatores metabólicos pré e pós-natais, distúrbios endócrinos hereditários, como a resistência à insulina e o diabetes tipo II, e fatores ambientais (dieta, atividade física, sono, intestino).

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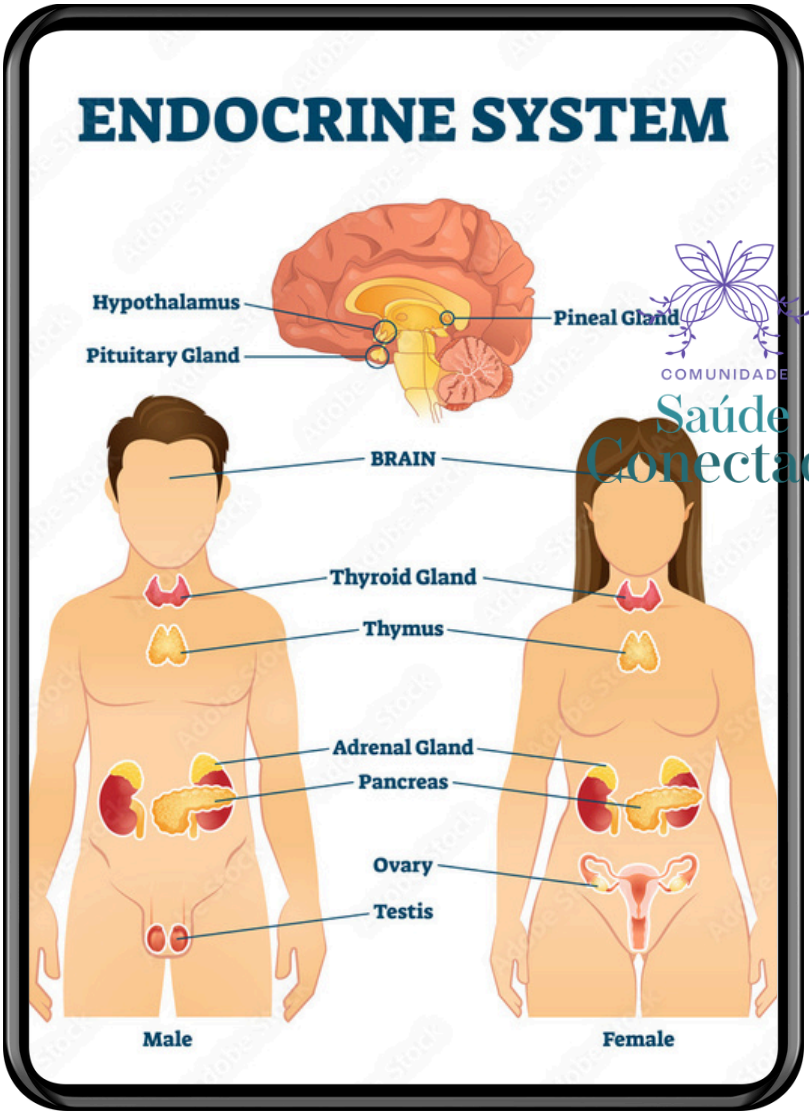

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ALTERAÇÕES HORMONAIS

- DIMINUICAO DE FSH
- AUMENTO DE LH, TESTOSTERONA, ANDROSTENEDIONA, DHEA
- LH/FSH > 3 – 50 A 88 %
- AUMENTO DE VOLUME OVARIANO

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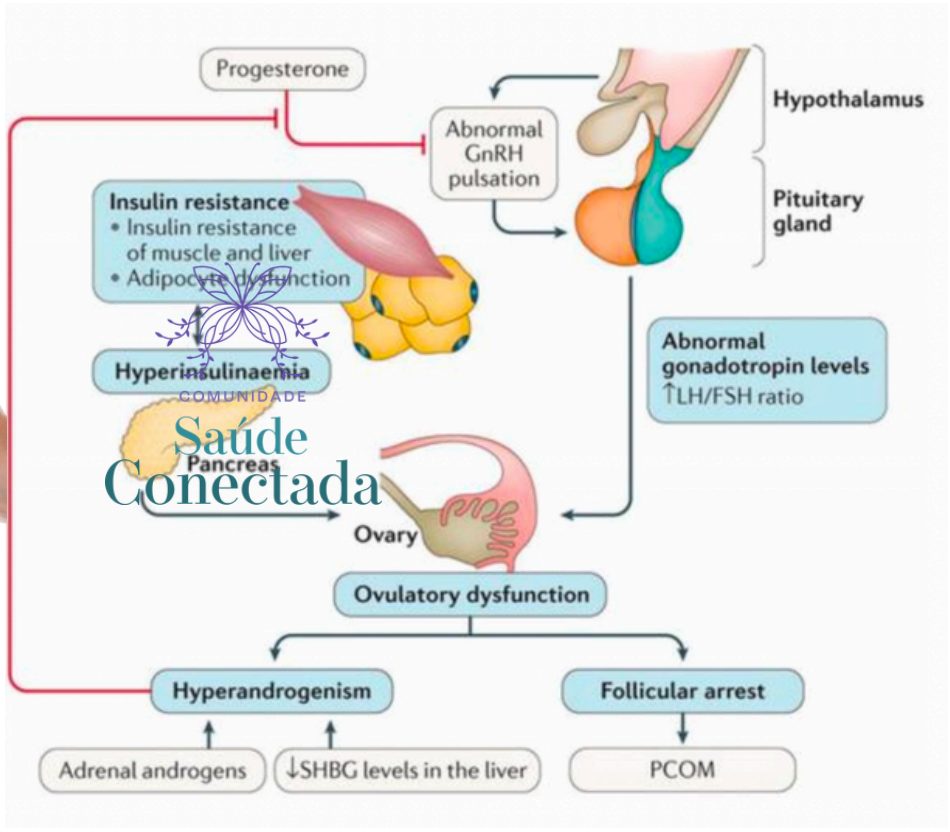

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Diagnóstico diferencial

Quadro 1. Diagnósticos diferenciais dos quadros de hiperandrogenismo

Diagnósticos diferenciais	Propedêutica complementar
Hiperprolactinemia	Dosagem de prolactina – se elevada, repetir em nova amostra.
Disfunções tireoidianas	Dosagem de TSH – se alterada, repetir a dosagem em nova amostra, juntamente com T4 livre.
Tumor ovariano produtor de androgênio	Dosagem de testosterona total ou livre (preferencialmente realizada por técnica de espectrometria de massa).
Tumor adrenal	Dosagem de deidroepiandrosterona sulfatada (DHEA-S).
Hiperplasia adrenal congênita (HAC)	Dosagem de 17-alfa-hidroxiprogesterona (17OHP), na fase folicular do ciclo. Em casos suspeitos, confirma-se o diagnóstico por meio do teste da cortrosina.

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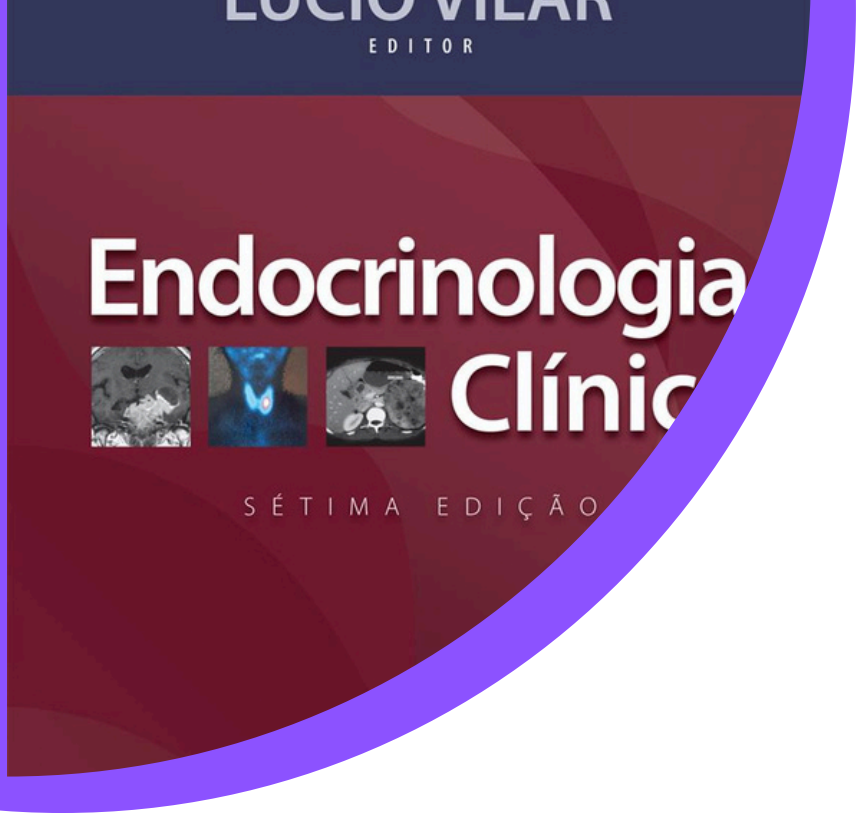

Tratamento

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Modificação do estilo de vida - alimentação, suplementação, exercicio fisico
Contraceptivos orais*
Acetato de ciproterona
Finasterida
Flutamida
Espirinolactona - suprime os andrógenos, melhora hirsutismo e acne. Mas pode impedir ovulação saudável e alterar o eixo HPA. Quando parar de tomar a ace pode ser ainda pior.
Metformina : boa ideia, atenção para deficiência de b12 e b9
Glitazonas
Citrato de Clomifeno
Gonadotrofinas
Cirurgia
Outros

Obs: análogos GLP-1.

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De onde vem o SOP ?

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Review Article

Polycystic Ovary Syndrome May Be an Autoimmune Disorder

Os fatores genéticos e hormonais desempenham papel crucial na patogênese da SOP.

O baixo nível de **progesterona** em PCOS causa “overstimulation” do sistema imunológico que produz mais estrogênio o que leva a vários auto-anticorpos.

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Macut D, Pfeifer M, Yildiz BO, Diamanti-Kandarakis E (eds): Polycystic Ovary Syndrome. Novel Insights into Causes and Therapy. Front Horm Res. Basel, Karger, 2013, vol 40, pp 51–63 (DOI: 10.1159/000341683)

Dyslipidemia and Oxidative Stress in PCOS

Djuro Macut[a] · Jelica Bjekić-Macut[b] · Ana Savić-Radojević[c]

[a]Clinic for Endocrinology, Diabetes and Metabolic Diseases, [b]CHC Bežanijska kosa, [c]Institute of Medical and Clinical Biochemistry, Faculty of Medicine, University of Belgrade, Belgrade, Serbia

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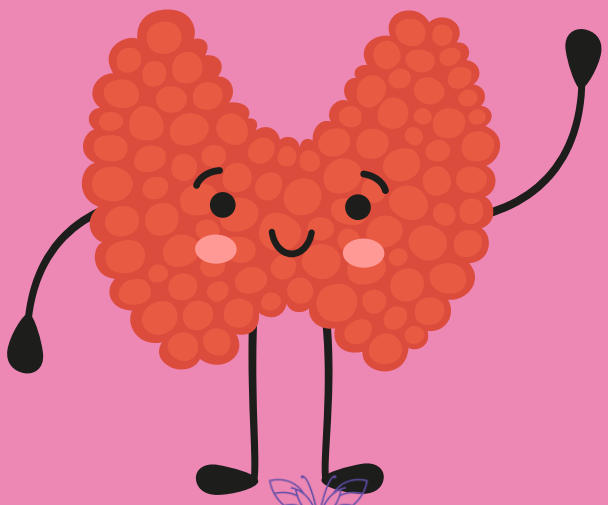

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Int J Clin Exp Med 2013;6(10):880-889
www.ijcem.com /ISSN:1940-5901/IJCEM1309028

Original Article

The relationship between thyroiditis and polycystic ovary syndrome: a meta-analysis

Danfeng Du, Xuelian Li

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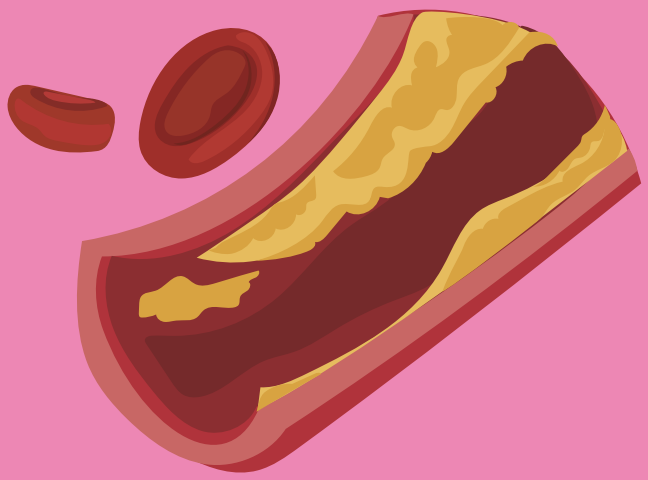

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J HEALTH POPUL NUTR 2015 Mar;33(1):157-167
ISSN 1606-0997 | $ 5.00+0.20

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Association of Insulin Resistance with Lipid Profile, Metabolic Syndrome, and Hormonal Aberrations in Overweight or Obese Women with Polycystic Ovary Syndrome

Mehranghiz Ebrahimi-Mamaghani[1], Maryam Saghafi-Asl[2], Saeed Pirouzpanah[2], Akbar Aliasgharzadeh[3], Soudabeh Aliashrafi[4], Niloufar Rezayi[4], Mahzad Mehrzad-Sadaghiani[5]

Os achados deste estudo indicaram que anormalidades lipídicas podem ocorrer na SOP, independentemente da RI.

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MEV

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Randomized Controlled Trial > J Clin Endocrinol Metab. 2015 Nov;100(11):4048-58.
doi: 10.1210/jc.2015-2778. Epub 2015 Sep 24.

Randomized Controlled Trial of Preconception Interventions in Infertile Women With Polycystic Ovary Syndrome

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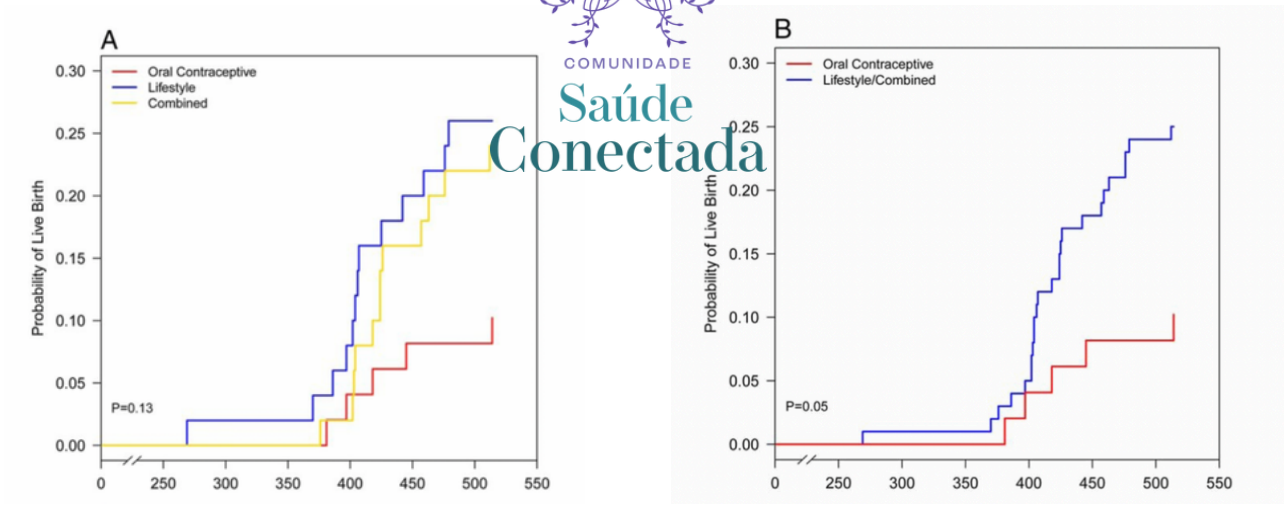

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DIETA ANTI INFLAMATÓRIA

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North American Journal of Medical Sciences

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N Am J Med Sci. 2015 Jul; 7(7): 310–316.
doi: 10.4103/1947-2714.161246

PMCID: PMC4525389

Anti-Inflammatory Dietary Combo in Overweight and Obese Women with Polycystic Ovary Syndrome

Amany Alsayed Salama, Ezzat Khamis Amine, Hesham Abd Elfattah Salem, and Nesrin Kamal Abd El Fattah

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In PCOS, a dietary trigger such as glucose is capable of inducing oxidative stress to stimulate an inflammatory response even in the absence of excess adiposity. Hyperandrogenism may be the originator of chronic low-grade inflammation. Diet-induced inflammation in particular may be the underpinning of insulin resistance in the disorder. Inflammation directly stimulates excess ovarian androgen production. Increased abdominal adiposity contributes to the inflammatory load in PCOS, and its development may be controlled by the severity of hyperandrogenism. Excess androgens encourage insulin resistance, leading to elevated insulin levels, which in turn stimulate further androgen synthesis. This vicious cycle results in a "snowball effect" worsening PCOS symptoms and making sufferers, especially susceptible to obesity and diabetes.[8,9]

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Dietary treatment

We adopted a Mediterranean-inspired low glycemic load anti-inflammatory diet based on combinations of nutrients (a combo diet) encouraging the consumption of legumes, fish, and low-fat dairy products in a Mediterranean context; with a composition of 25% proteins, 25% fat, and 50% carbohydrates. Diets were designed as reduced-energy, low-fat, low-saturated fat, and moderate-to-high fiber diets. The GL was reduced by lowering sugar content in favor of more complex carbohydrates. Estimated energy requirements were calculated for each subject according to the Institute of Medicine Equation, then subtracting 500 kcal. Meal plans were previously designed using the Diabetic Exchange Calculation Forum; combo diets of 2,000; 1,800; 1,600; 1,400; 1200; 1,000; and 800 kcal content were prepared with multiple options to suit a variety of personal taste and preference,[11] using the Diabetic Exchange List. By evenly spacing carbohydrate foods through the day and by eating about the same amount at each meal or snack, blood glucose control stays within targets. We encouraged the consumption of a multigrain commercial bread brand (Rich Bake multigrain toast - six grains), a low GI bread. Menu plans and shopping lists were provided at each visit. They were directed to consume unsaturated sources of fat such as flaxseeds (40 g/day),[12,13,14] and olive oil while limiting overall fat intake.

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- Dieta do mediterrâneo
- Sementes
- Azeite

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Due to cultural, environmental, and sometimes religious barriers; we instructed all participants to use the stairs to the upper floor up and down for 30 min/day, and three times 10 min/day of sit-ups or abdominal crunches. Red meat was limited to once every 2 weeks, chickens once weekly, and fish at least twice weekly; and we encouraged the consumption of legumes. As consuming small, frequent meals rather than large, less frequent meals has been associated with improved glycemic control and lipid profiles; five small meals, 3 h apart were constructed. Ginger, chili peppers, black pepper, curcumin, bay leaves, fennel, anise, caraway, cumin, coriander, clove, cinnamon, marjoram, rosemary, and thyme were recommended for

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N Am J Med Sci

Table 2

Metabolic, hormonal, and inflammatory markers before and after intervention

Parameter	Before		After		W
	Mean	SD	Mean	SD	
Fasting blood glucose (FBG; mg/dL)	91.87	10.36	86.43	9.05	
Fasting insulin (FI; µIU/mL)	14.80	8.50	9.65	5.33	
Homeostatic model assessment (HOMA) index	1.90	1.09	1.25	0.68	
Total cholesterol (TCh; mg/dL)	198.68	38.71	178.13	29.06	
High-density lipoprotein cholesterol (HDL-C; mg/dL)	47.95	11.27	49.19	9.15	
Low-density lipoprotein cholesterol (LDL-C; mg/dL)	130.11	36.29	112.83	27.69	
Triglycerides (TG; mg/dL)	104.61	36.70	79.08	26.90	
TG/HDL	2.35	1.20	1.68	0.80	
Total testosterone (TT; ng/mL)	0.39	0.18	0.36	0.15	
Free testosterone (FT; pg/mL)	5.87	3.15	4.19	1.97	
Sex hormone binding globulin (SHBG; nmol/L)	47.90	25.23	72.03	36.07	
Free androgen index (FAI)	3.54	2.63	2.10	1.11	
C-reactive protein (CRP; mg/L)	9.57	9.43	5.09	3.76	
Serum amyloid A (SAA; mg/L)	19.57	24.36	10.00	12.37	

The 0.05 level was used as the cut off value for statistical significance. SD = Standard deviation

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GLICAÇÃO AVANÇADA

Açúcar

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Review

Advanced glycation end-products: modifiable environmental factors profoundly mediate insulin resistance

Mona S. Ottum and Anahita M. Mistry*

Dietetics and Human Nutrition Program, 318 Marshall Building, Eastern Michigan University, Ypsilanti, MI 48197, USA

Advanced glycation end-products are toxic by-products of metabolism and are also acquired from high-temperature processed foods. They promote oxidative damage to proteins, lipids and nucleotides. Aging and chronic diseases are strongly associated with markers for oxidative stress, especially advanced glycation end-products, and resistance to peripheral insulin-mediated glucose uptake. Modifiable environmental factors including high levels of refined and simple carbohydrate diets, hypercaloric diets and sedentary lifestyles drive endogenous formation of advanced glycation end-products via accumulation of highly reactive glycolysis intermediates and activation of the polyol/aldose reductase pathway producing high intracellular fructose. High advanced glycation end-products overwhelm innate defenses of enzymes and receptor-mediated endocytosis and promote cell damage via the pro-inflammatory and pro-oxidant receptor for advanced glycation end-products. Oxidative stress disturbs cell signal transduction, especially insulin-mediated metabolic responses. Here we review emerging evidence that restriction of dietary advanced glycation end-products significantly reduces total systemic load and insulin resistance in animals and humans in diabetes, polycystic ovary syndrome, healthy populations and dementia. Of clinical importance, this insulin sensitizing effect is independent of physical activity, caloric intake and adiposity level.

Key Words: oxidative stress, insulin resistance, glycation, AGEs, Western diet

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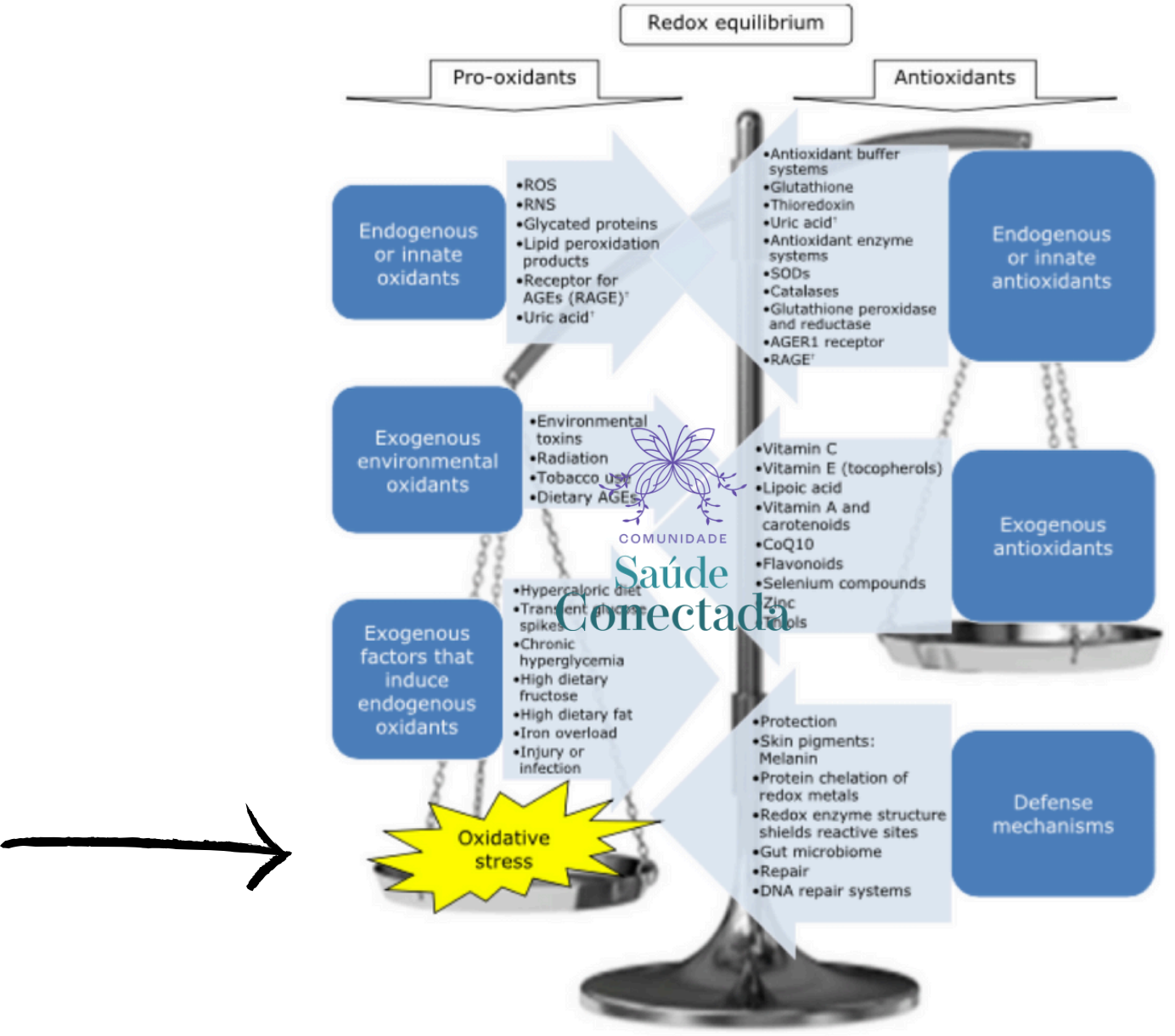

Fig. 1. Redox Homeostasis and Oxidative Stress. Many known factors contribute to redox homeostasis in human physiology. On the left are those that increase oxidation with opposing anti-oxidant mechanisms on the right. *Uric acid and RAGE are antioxidants at low concentrations and pro-oxidant and markers of oxidative stress at high concentrations.

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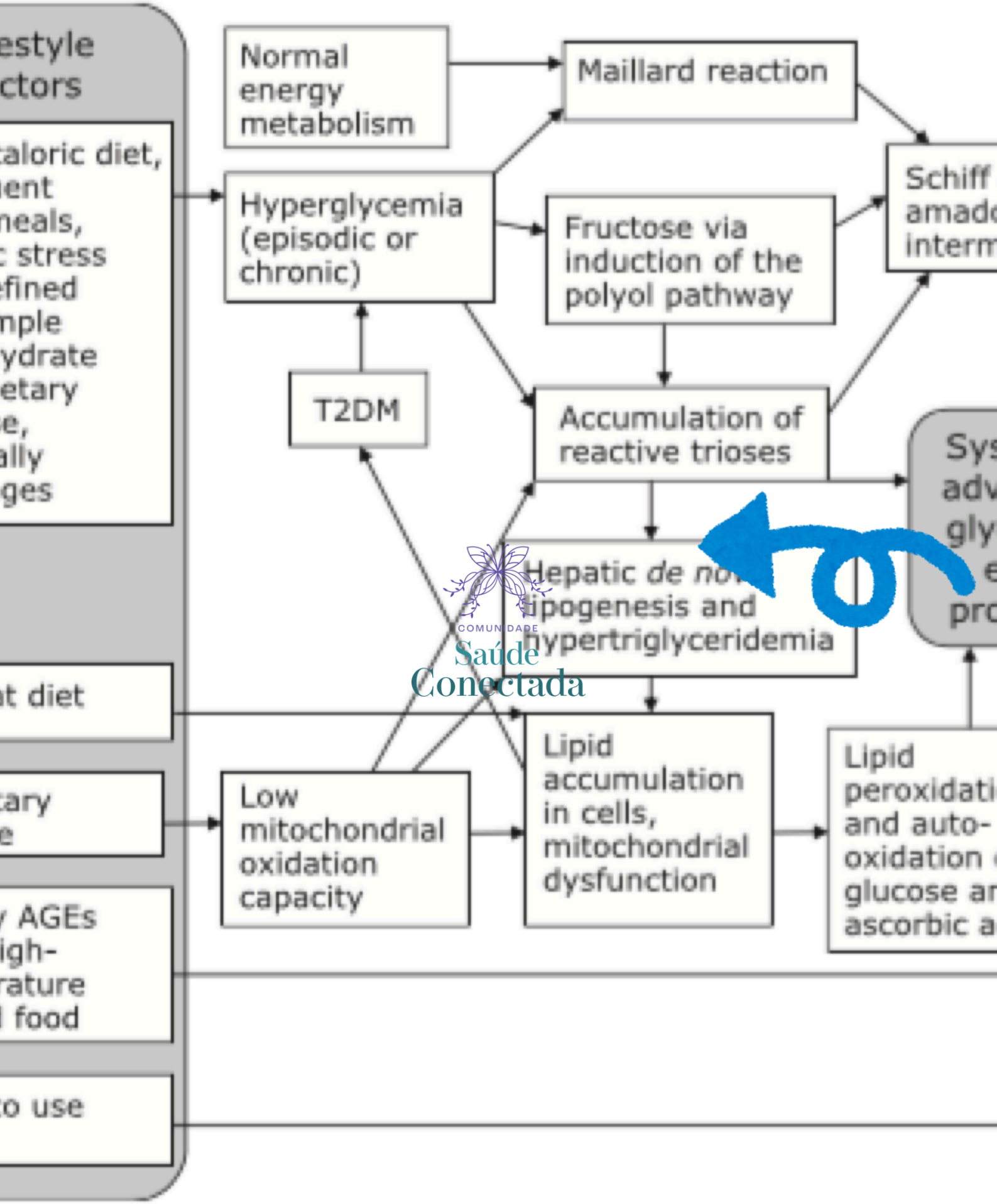

Known pathways to physiologic load of toxic advanced glycation end-p
s formation while dietary AGEs enter the system pre-formed with heat-treat

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Iran J Reprod Med Vol. 13. No. 1. pp: 1-8, January 2015

Systematic review

Antioxidants and management of polycystic ovary syndrome in Iran: A systematic review of clinical trials

Leila Amini[1] Ph.D. candidate, Najmeh Tehranian[1] Ph.D., Mansoureh Movahedin[2] Ph.D., Fahimeh Ramezani Tehrani[3] M.D., Saeedeh Ziaee[1] M.D.

Abstract

Background: Recently there is a focus on the antioxidants as adjuvant treatment of polycystic ovary syndrome (PCOS), the most endocrinopathy in reproductive age women.

Objective: The aim of this review is answer to the question whether antioxidants are effective for managing of hormonal and metabolic problems in women with PCOS based on first degree evidences from Iran.

Materials and Methods: A systematic review of clinical trials was done in Persian and international databases including PubMed, Scientific Information Database, Google Scholar, Iran Medex, and Magiran up to 2013. Keywords were including polycystic ovary syndrome, Iran, vitamin, antioxidant. From 440 potential studies found electronically, 11 studies; including 444 women in intervention and 390 women in control groups. Intervention in three studies was Calcium-vitamin D or calcitriol; in three studies was ω-3 fatty acids; in two studies was N-acetyl cysteine; in one study was folic acid; in one study was Zinc; and in one study was Soy.

Results: Finally, 11 studies that were relevant and met the inclusion criteria reviewed. There were 7 studies in English and 4 studies in Persian. We couldn't include all studies because all full texts were not accessible.

Conclusion: The results showed that antioxidants and vitamins have positive effects on management of PCOS women. Although it seems more studies is necessary in this field.

Key words: *Polycystic ovary syndrome, Iran, Vitamin, Antioxidant.*

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Table I. Information about reviewed studies

Author/ Language (ref)	Year	Place	Type of Study	Sample Size	Intervention	Int. Duration	Main Outcomes
Khani et al English (24)	2011	Isfahan	Quasi-randomized trial	137	1-Soy (Genistein 18 mg /BD) 2-Placebo	3 months	**Reduce:** LH, TG, LDL, DHEAS, and T
Kazemoni et al English (25)	2008	Shiraz	Prospective clinical trial	70	Folic acid 1mg/d	3 months	**Reduce:** Homocysteine (HCY) in hyperhomocysteinemic PCOS women
Salehpour et al English (26)	2012	Tehran	Placebo-controlled double-blind randomized clinical trial	180	1-N-Acetyl Cysteine (NAC) 1.2 g/d +100 mg Clomiphene Citrate (CC) 2- CC+ placebo	5 days	**Increase:** Number of follicles greater than18 mm, Endometrial thickness on the day of hCG administration, Ovulation and pregnancy rates, No adverse side-effects and ovarian hyperstimulation syndrome
Salehpour et al English (27)	2009	Tehran	Prospective double-blind randomized clinical trial	46	1-N-Acetyl Cysteine (NAC) 1.8 g/d 2- placebo	6 weeks	**Increase:** Ovulation rate, HDL **Reduce:** Weight and BMI,Waist/hip ratio, FBS and Insulin, TC, LDL, HOMA-IR A
Rashidi et al English (28)	2009	Tehran	Randomized clinical trial	150	1- Calcium-D 2- Calcium-D+ metformin 1500 mg/d 3-metformin	3 months	**Increase (in group 2):** Number of dominant Follicles ≥ 14 mm, regular menses
Bonakdaran et al Persian (31)	2012	Mashhad	Randomized clinical trial	51	1-Metformin 1000 mg/day 2- Calcitriol 0.5 mic/d 3-Placebo	3 months	**Metformin improved:** Weight, BMI, Insulin, Insulin resistance. **Calcitriol improved:** PTH, Systolic Blood pressure, Ovulation
Mohammad Beigi et al Persian (32)	2012	Kermanshah	Randomized control clinical trial	44	1-Vit D3 400 IU/d+ Calcium 1000 mg/d + EPA 50 mg/d + DHA 50 mg/d +placebo	3 months	**Increase:** Median and mean of follicular size
Mohammadi et al English (29)	2012	Tabriz	Double-blind randomized controlled clinical trial	64	1-omega -3 fatty acids 4 g/d 2-Placebo	8 weeks	**Reduce:** TC/HDL-C and LDL-C/HDL-C ratios **Increase:** PON1 activity
Raftar et al Persian (34)	2012	Tabriz	Double-blind randomized controlled clinical trial	64	1-omega -3 fatty acids 4 g/d 2-Placebo	8 weeks	**Reduce:** TC, TG, LDL, MDA **Increase:** HDL
Raftar et al Persian (33)	2012	Tabriz	Double-blind randomized controlled clinical trial	61	1-omega -3 fatty acids 4 g/d 2-Placebo	8 weeks	**Reduce:** Glucose, Insulin, and Insulin resistance
Pourseymour Fard Tabrizi et al English (30)	2010	Tabriz	Randomize double blind,Placebo controlled parallel groups trial	60	1-Zinc 50 mg/d 2-Placebo	8 weeks	**Reduce:** HOMA-IR score, Fasting TC, LDL-C, TG, T, TG/HDL-C ratio

LH= Luteinizing hormone
DHEAS=Dehydroepiandrosterone sulfate
FBS=Fasting blood sugar
PTH=Parathyroid hormone
HOMA-IR=Homeostatic model assessment- Insulin resistance index
TG=Triglyceride
T=Testosterone
TC=Total cholesterol
HDL=High density lipoprotein
LDL= Low density lipoprotein
BMI=Weight and Body mass index
MDA=Malondialdehyde
PON1=Paraoxonase 1

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THE JOURNAL OF
Obstetrics and Gynaecology Research
doi:10.1111/jog.12907 J. Obstet. Gynaecol. Res. Vol. 42, No. 3: 279–285, March 2016

Chromium picolinate reduces insulin resistance in polycystic ovary syndrome: Randomized controlled trial

Sherif Ashoush[1], Amgad Abou-Gamrah[1], Hassan Bayoumy[1] and Noura Othman[2]

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- 6 meses – 1000 mcg Pic. Cr.
- Foi associado com redução significativa do IMC e insulina.
- Aumentou significativamente as chances de ovulação e menstruação regular.

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Macut D, Pfeifer M, Yildiz BO, Diamanti-Kandarakis E (eds): Polycystic Ovary Syndrome. Novel Insights into Causes and Therapy. Front Horm Res. Basel, Karger, 2013, vol 40, pp 51–63 (DOI: 10.1159/000341683)

Dyslipidemia and Oxidative Stress in PCOS

Djuro Macut[a] · Jelica Bjekić-Macut[b] · Ana Savić-Radojević[c]

[a]Clinic for Endocrinology, Diabetes and Metabolic Diseases, [b]CHC Bežanijska kosa, [c]Institute of Medical and Clinical Biochemistry, Faculty of Medicine, University of Belgrade, Belgrade, Serbia

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Suplementação com ômega 3, ácido alfa lipóico e N acetilcisteina (NAC) podem ser considerados uma boa estratégia anti - inflamatória e antioxidante para o SOP

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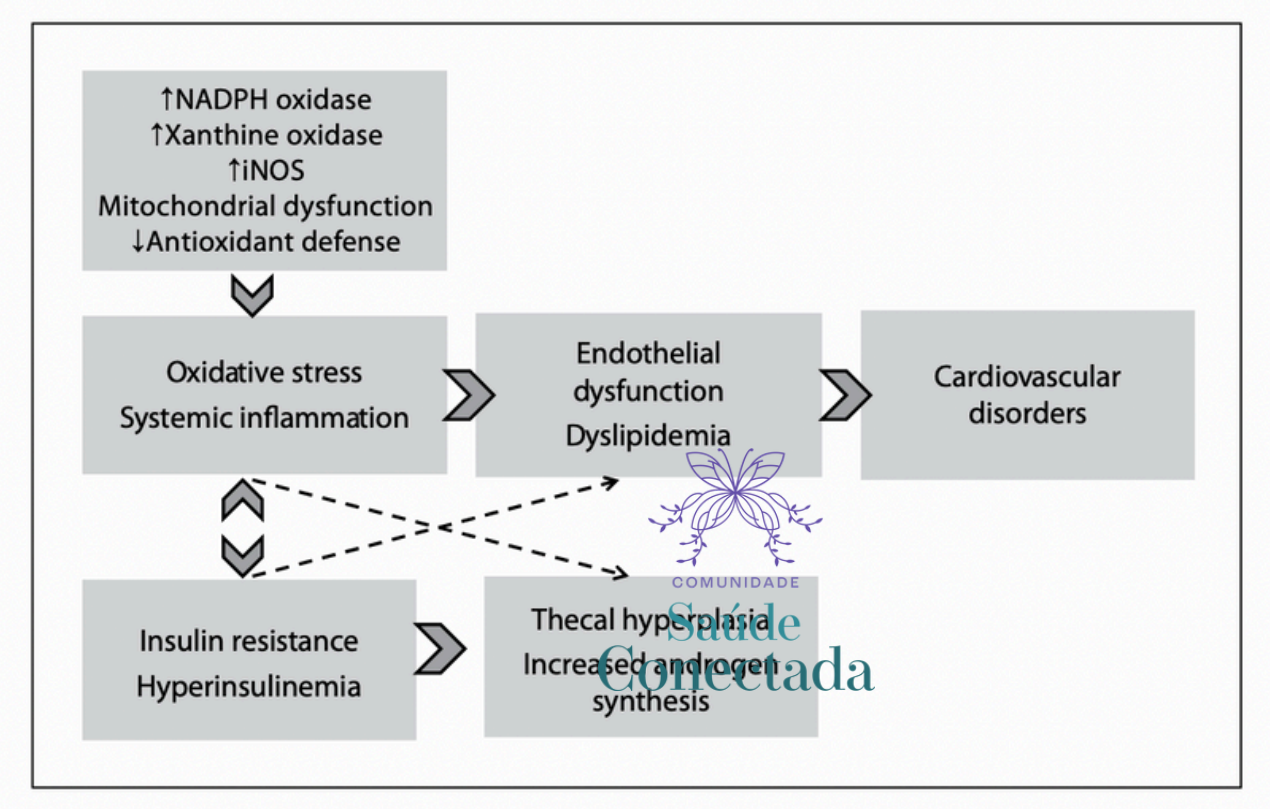

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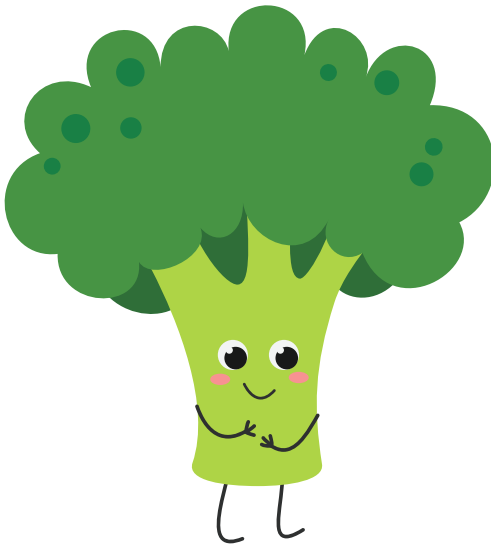

Antioxidantes:

- Vit C, vit E, carotenóides, polifenoós
- Coq10
- N acetil cisteina, glutationa
- Isotiocianatos

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Randomized Controlled Trial > Phytother Res. 2010 Feb;24(2):186-8. doi: 10.1002/ptr.2900.

Spearmint herbal tea has significant anti-androgen effects in polycystic ovarian syndrome. A randomized controlled trial

Paul Grant

Affiliations + expand

PMID: 19585478 DOI: 10.1002/ptr.2900

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Clinical Trial > Phytother Res. 2007 May;21(5):444-7. doi: 10.1002/ptr.2074.

Effect of spearmint (Mentha spicata Labiatae) teas on androgen levels in women with hirsutism

Mehmet Akdoğan [1], Mehmet Numan Tamer, Erkan Cüre, Medine Cumhur Cüre, Banu Kale Köroğlu, Namik Delibaş

250 ml - 2 x ao dia

5 g de ervas secas (infusão por 5 a 10 min)

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GYNECOLOGICAL ENDOCRINOLOGY
http://informahealthcare.com/gye
ISSN: 0951-3590 (print), 1473-0766 (electronic)
Gynecol Endocrinol, Early Online: 1–4
© 2016 Informa UK Limited, trading as Taylor & Francis Group. DOI: 10.1080/09513590.2016.1259407
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Taylor & Francis Group

ORIGINAL ARTICLE

Substituting poly- and mono-unsaturated fat for dietary carbohydrate reduces hyperinsulinemia in women with polycystic ovary syndrome

Dalia Perelman, Nicole Coghlan*, Cindy Lamendola, Susan Carter, Fahim Abbasi, and Tracey McLaughlin

Division of Endocrinology, Stanford University School of Medicine, Stanford, CA, USA

Monoinsaturada

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Polinsaturada

sementes de chia

sementes de linhaça

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Original Article

Effect of Omega-3 Supplementation on Visfatin, Adiponectin, and Anthropometric Indices in Women with Polycystic Ovarian Syndrome

Azadeh Nadjarzadeh [1, 2], Razieh Dehghani-Firouzabadi [3], Hoorieh Daneshbodi [4*], Mohammad Hassan Lotfi [5], Niloofar Vaziri [4], Hassan Mozaffari-Khosravi [2]

Abstract

Background: Polycystic ovary syndrome (PCOS) is a multifactorial, metabolic disorder. Characteristics are chronic anovulation, polycystic ovaries and hyperandrogenism. The aim of this study was to determine the effect of omega-3 supplementation on visfatin, adiponectin, and anthropometric indices in PCOS women.

Methods: The study was a randomized double blind placebo-controlled clinical trial. It was conducted on 84 women with polycystic ovary syndrome (26.92±5.05 years, BMI=31.69 Kg/m^2) who referred to the fertility and infertility research center and Shahid Sadoughi hospital in Yazd. After the examination, evaluation and paramedical assessment by obstetrician, they were recruited. They took 3 capsules of omega-3 (each one contained 180 *mg* EPA and 120 *mg* DHA) or placebo (each contained 1 *g* paraffin) daily for 8 weeks. Statistical analysis was paired T-test and student T-test, and a $p<0.05$ was considered statistically significant.

Results: After the intervention, visfatin concentration did not change in neither groups. But, at the end of the study, the mean of adiponectin concentration increased ($p<0.001$) in omega-3 group. Moreover, the mean of changes in this factor was significantly different between groups ($p<0.005$). FSH did not change in two groups of the study. However, the mean of LH decreased about 1.74 *mIU/ml* in omega-3 group ($p<0.005$). The mean of change of LH/FSH ratio between groups was significant ($p<0.05$). After the intervention, prolactin did not meaningfully change in both groups.

Conclusion: Our results showed that 8 weeks of supplementation of omega-3 may have some beneficial effects on PCOS biochemical characteristics such as LH, LH/FSH, and adiponectin.

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Accepted Manuscript

The effects of omega-3 fatty acids and vitamin E co-supplementation on gene expression of lipoprotein(a) and oxidized low-density lipoprotein, lipid profiles and biomarkers of oxidative stress in patients with polycystic ovary syndrome

Elham Rahmani, Mansooreh S... h
Foroozanfard, Shahnaz Ah... hadavod

2.4. Intervention

In the treatment gr... from flaxseed oil containing 400 m... ts for 12 weeks. Supplements and p... ufactured by Barij Essence (Kashan, Ira...

1000 mg Omega 3
400 UI Vit. E
Conectada

Overall, omega-3 fatty acids and ... supplementation for 12 weeks in PCOS women significantly improved gene expression of Lp(a) and Ox-LDL, lipid profiles and biomarkers of oxidative stress.

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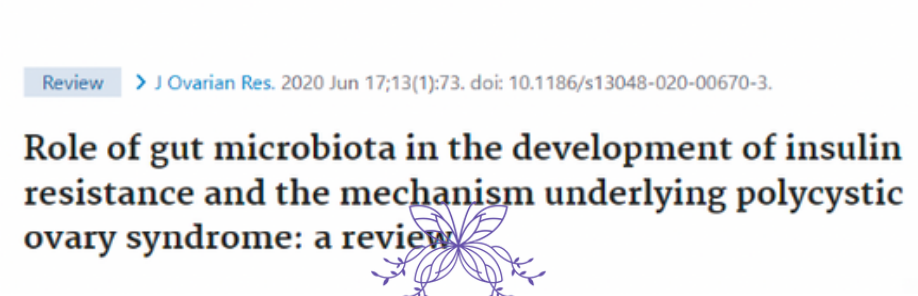

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Pesquisas apontam que o **aumento da permeabilidade intestinal**, e consequentemente, da passagem de LPS (toxina bacteriana) resulta na ativação de receptores que estimulam a liberação de substâncias **inflamatórias**, que alteram a resposta do receptor de insulina, e consequentemente contribui para **RESISTÊNCIA A INSULINA**.

Ou seja, a disbiose intestinal contribui para o início e a manutenção da resistência à insulina.

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RESEARCH ARTICLE

Alterations in Gut Microbiome Composition and Barrier Function Are Associated with Reproductive and Metabolic Defects in Women with Polycystic Ovary Syndrome (PCOS): A Pilot Study

Lisa Lindheim[1]*, Mina Bashir[1], Julia Münzker[1], Christian Trummer[1], Verena Zachhuber[1], Bettina Leber[2,3], Angela Horvath[4], Thomas R. Pieber[1,3], Gregor Gorkiewicz[5,6], Vanessa Stadlbauer[3,4], Barbara Obermayer-Pietsch[1,3]

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Disfunção Barreira Intestinal e Endotoxemia pode contribuir para o aparecimento de SOP

Conclusion

Patients with PCOS have a lower diversity and an altered phylogenetic profile in their stool microbiome, which is associated with clinical parameters. Gut barrier dysfunction and endotoxemia were not driving factors in this patient cohort, but may contribute to the clinical phenotype in certain PCOS patients.

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1) A **inflamação** interfere na função do receptor de insulina, causando **hiperinsulinemia**, o que aumenta a produção de andrógeno ovariano e impede o desenvolvimento de folículos normais (com a testosterona livre em excesso, que impede a mulher de ovular e menstruar, leva à acne e pelos em excesso).

2) Além disso, a inativação inadequada do sistema imunológico pode explicar a maior associação entre mulheres com SOP e tireoidite autoimune como a de Hashimoto.

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3) Já sabemos que a microbiota intestinal influencia a secreção de peptídeos intestinais e cerebrais, como a grelina, GLP-1, serotonina, ácidos biliares, etc. Uma disbiose pode contribuir para alteração na quantidade destes peptídeos, influenciando no eixo intestino-cérebro (alterações do humor, depressão, ansiedade, compulsão alimentar) e no metabolismo, contribuindo de forma negativa na SOP.

4)Por sua vez, os andrógenos (produzidos pelos ovários e glândulas adrenais estimulados pela hiperinsulinemia) alteram a composição da microbiota intestinal, perpetuando a disbiose e consequentemente o desenvolvimento da SOP, criando-se um ciclo vicioso.

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Effects of Probiotic Supplementation on Pancreatic β-cell Function and C-reactive Protein in Women with Polycystic Ovary Syndrome: A Randomized Double-blind Placebo-controlled Clinical Trial

Tanaz Shoaei, Motahar Heidari-Beni, Hatav Ghasemi Tehrani,[1] Awat feizi,[2] Ahmad Esmaillzadeh, and Gholamreza Askari

Conecta Saúde

Patients in probiotic group received one Familact probiotic capsule (500 mg) daily that each capsule contained the following bacterial strains: *Lactobacillus casei* 7×10^9 CFU/g, *Lactobacillus acidophilus* 2×10^9 CFU/g, *Lactobacillus rhamnosus* 1.5×10^9 CFU/g, *Lactobacillus bulgaricus* 2×10^8 CFU/g,*Bifidobacterium breve* 2×10^{10} CFU/g, *Bifidobacterium longum* 7×10^9 CFU/g, *Streptococcus thermophiles* 1.5×10^9 CFU/g. Participants in the placebo group received the placebo that contained

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DISCUSSION

The current study demonstrated that taking probiotic supplements for 8-week had no significant effect on CRP and pancreatic β-cell function among PCOS women. However, after adjustment with some covariates, only serum insulin level decreased significantly in the probiotic group compare with placebo group. Previous studies have assessed the effect of probiotic supplementation on insulin resistance and inflammation marker. To the best of our knowledge, the present study is the first study that investigated the effect of probiotic supplementation on pancreatic β-cell function and CRP in PCOS women.

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CONCLUSIONS

Probiotic supplementation for 8-week had no beneficial effects on FBS, HOMA-IR, QUICKI and CRP significantly. After adjustment with some covariates, serum insulin changes were significantly different between groups.

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Original Article

Effects of Ramadan Fasting on Glucose Homeostasis, Lipid Profiles, Inflammation and Oxidative Stress in Women with Polycystic Ovary Syndrome in Kashan, Iran

Zatollah Asemi PhD[1], Mansooreh Samimi MD[2], Mohsen Taghizadeh PhD[1], Ahmad Esmaillzadeh PhD•[3,4,5]

Abstract

Background: To our knowledge, no reports are available indicating the effects of Ramadan fasting on metabolic parameters, inflammatory factors and oxidative stress in polycystic ovary syndrome (PCOS). The current study was designed to evaluate the effects of Ramadan fasting on metabolic status among women with PCOS.

Methods: This cross-sectional study was conducted on twenty seven PCOS patients who had fasted for a mean period of 16.5 hours a day during the 29 days of the month of Ramadan in Kashan, Iran. Fasting blood samples were collected at the beginning of the study and after 29 days of the study to quantify related variables. To identify within-group differences (before and after Ramadan), paired-samples t-tests were used.

Results: Plasma nitric oxide (NO) levels in PCOS women after Ramadan fasting were significantly higher compared to the baseline values (70.63 ± 15.78 vs. 59.94 ± 13.87 µmol/L, $P = 0.003$). Post-Ramadan levels of plasma glutathione (GSH) increased significantly in comparison with pre-Ramadan (974.95 ± 414.20 vs. 746.96 ± 205.93 µmol/L, $P = 0.011$). In addition, a trend toward a significant effect of Ramadan fasting on reducing serum high sensitivity C-reactive protein (hs-CRP) concentrations (2001.07 ± 1686.08 vs. 2962.72 ± 2845.21 ng/mL, $P = 0.072$) was seen. We did not observe any significant effect of Ramadan fasting on glucose hemostasis parameters, lipid profiles or total antioxidant capacity (TAC).

Conclusion: In conclusion, Ramadan fasting in women with PCOS for 4 weeks had beneficial effects on NO and GSH levels, but did not affect glucose hemostasis parameters, lipid profiles or TAC.

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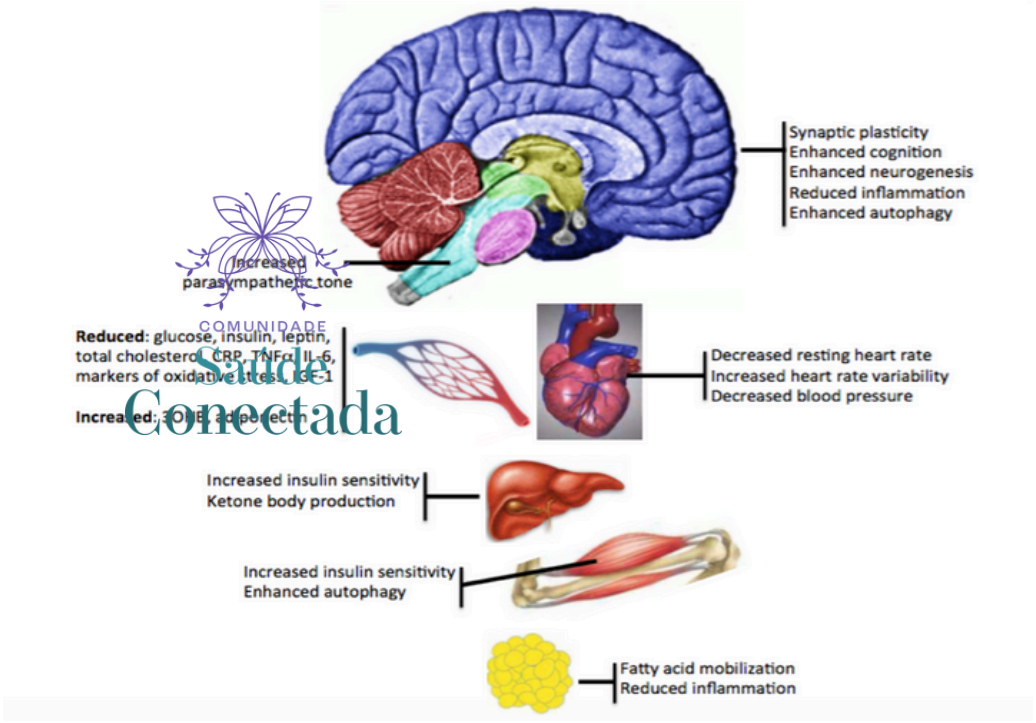
Accepted Manuscript

Title: Impact of Intermittent Fasting on Health and Disease Processes

Author: Mark P. Mattson Valter D. Longo Michelle Harvie

PII: S1568-1637(16)30251-3
DOI: http://dx.doi.org/doi:10.1016/j.arr.2016.10.005
Reference: ARR 717



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Accepted Manuscript

Binge eating behaviours and food cravings in women with Polycystic Ovary Syndrome

Y.M. Jeanes, S. Reeves, E.L. Gibson, C. Piggott, V.A. May, K.H. Hart

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Prevalência da compulsao alimentar
27 obesas e 28 magras
70 % obesas, classificadas – 39 % comportamento clinicamente significativo.
Mulheres magras com SOP apresentaram escores de sintomas de compulsão alimentar significativamente maiores

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Os medicamentos de forma geral agem somente nos SINAIS E SINTOMAS e não tratam a CAUSA.

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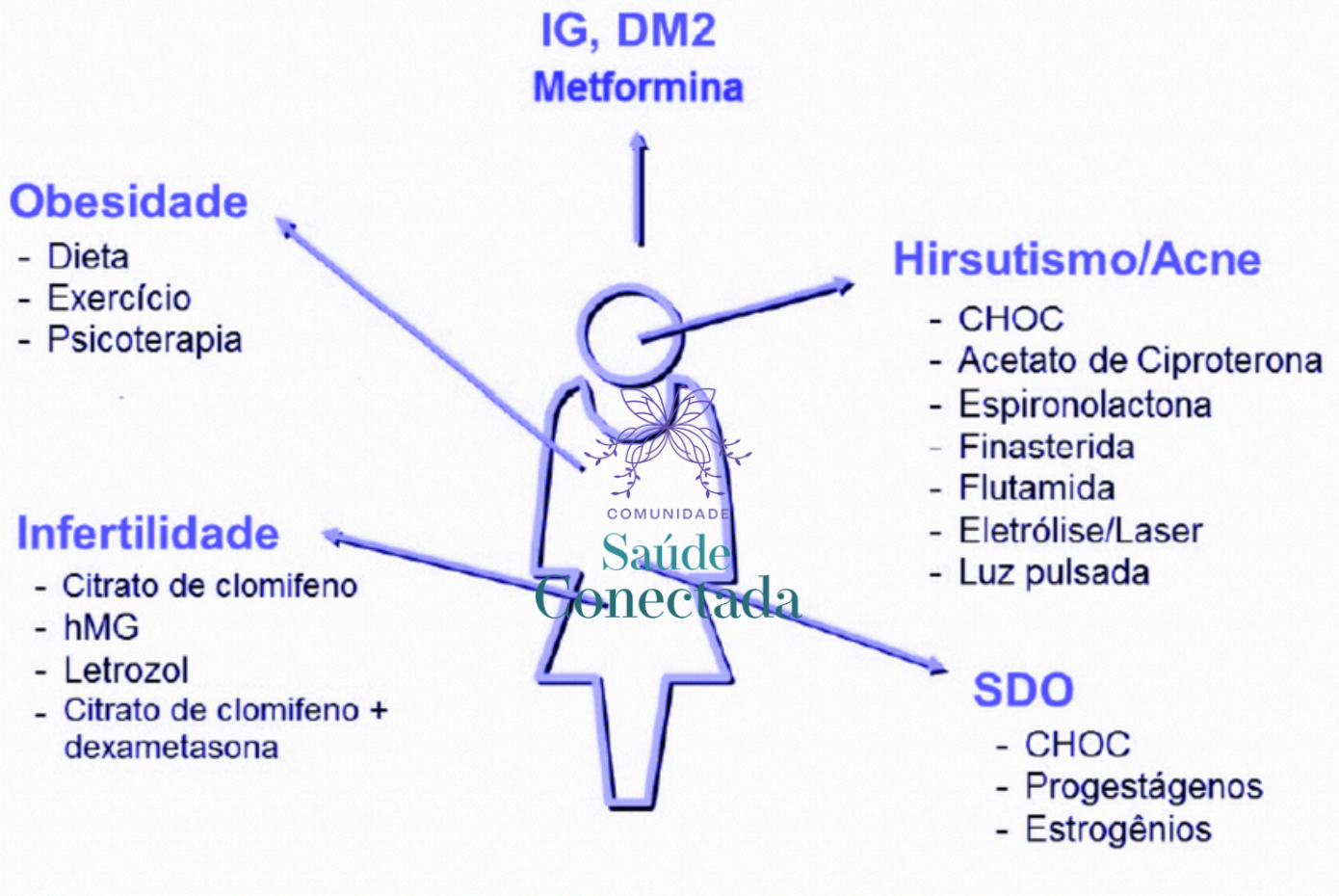

Figura 5. Tratamento da SOP. CC: citrato de clomifeno. CHOC: contraceptivos hormonais orais combinados. DM2: diabetes mellitus tipo 2. hMG: gonadotrofina menopausal humana. IG: intolerância à glicose. SDO: sangramento por disfunção ovulatória

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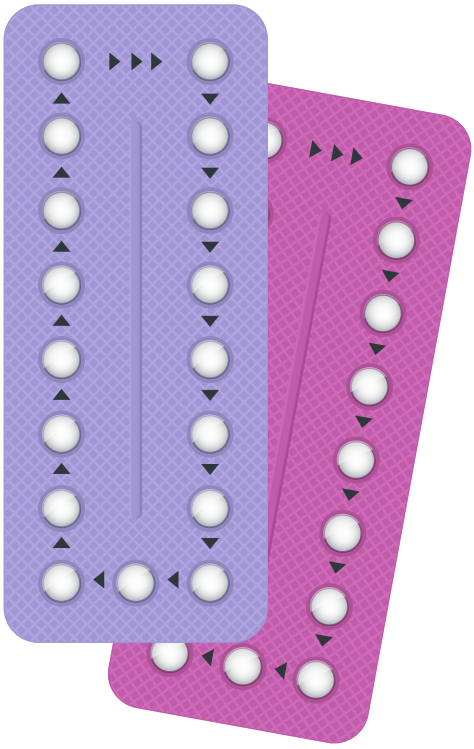

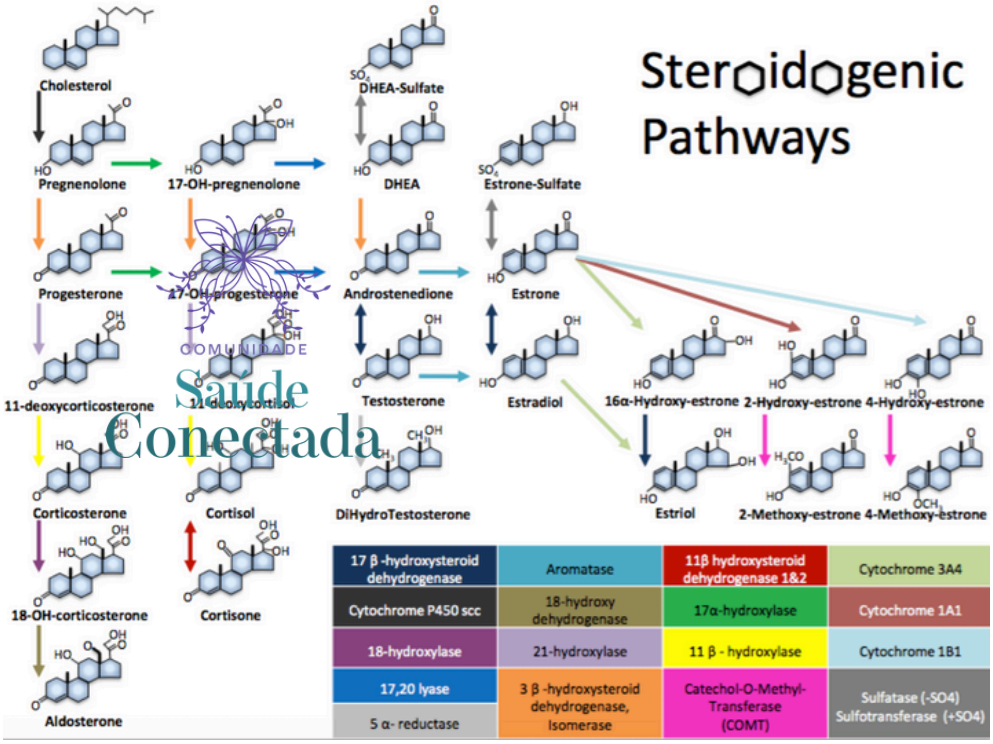

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O tratamento com AC por seis meses aumentou os níveis plasmáticos de **ICAM-1, MCP-1 e TNF** - entre as mulheres com SOP, embora os ACs ajudem significativamente a melhorar características do hiperandrogenismo e regularizar ciclos menstruais.

Estas **citocinas** se correlacionam positivamente com muitos parâmetros metabólicos, incluindo glicose plasmática, lipídios e HOMA-IR. Uma maior investigação com estudos longitudinais bem desenhados e randomizados pode ajudar a avaliar o impacto do AC no perfil proinflamatório entre as mulheres com PCOS.

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Review > Hum Reprod Update. 2015 Sep-Oct;21(5):560-74. doi: 10.1093/humupd/dmv025. Epub 2015 Jun 9.

Metformin and lifestyle modification in polycystic ovary syndrome: systematic review and meta-analysis

Negar Naderpoor [1], Soulmaz Shorakae [1], Barbora de Courten [1], Marie L Misso [2], Lisa J Moran [3], Helena J Teede [4]

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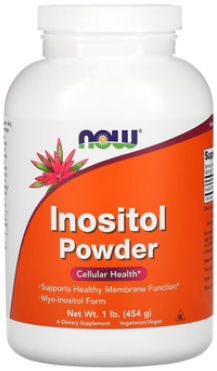

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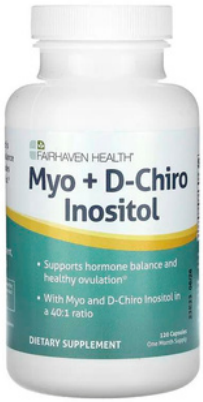
Randomized Controlled Trial > Gynecol Endocrinol. 2017 Jan;33(1):39-42.
doi: 10.1080/09513590.2016.1236078. Epub 2016 Nov 3.

Comparison of two insulin sensitizers, metformin and myo-inositol, in women with polycystic ovary syndrome (PCOS)

Franca Fruzzetti [1], Daria Perini [1], Marinella Russo [1], Fiorella Bucci [1], Angiolo Gadducci [1]



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STUDY PROTOCOL Open Access

Probiotic vs. placebo and metformin: probiotic dietary intervention in polycystic ovary syndrome – A randomized controlled trial

Valentin Borzan[1,2], Regina Riedl[3] and Barbara Obermayer-Pietsch[1,4*]

Abstract

Background Polycystic Ovary Syndrome (PCOS) is a very common endocrine disorder with a variety of symptoms. Current treatment options include the contraceptive pill as well as metformin, however both treatments are limited to specific symptoms and have common side effects.

Methods This phase IV study is a monocentric double blinded randomized clinical trial comparing the effects of six months of probiotic intervention to a placebo, with an additional open-label metformin arm as a positive control in a total of 180 participants with PCOS. The first of three visits is the screening visit, where inclusion/exclusion criteria are assessed. At the first visit, they are randomised into one of the three treatment arms equally and receive their study medication. After six months, all assessments from the first two visits are repeated. The primary endpoint is the change in free testosterone levels after the intervention, while secondary endpoints include changes in hormonal and metabolic parameters associated with PCOS as well as the gut microbial composition and diversity after intervention.

Discussion Based on new insights into the role of the gut microbiome in PCOS development, this study is exploring the potential of using probiotics to treat women with PCOS symptoms. If successful, this new therapy approach could open a new realm of possibilities for treating PCOS. To our knowledge, this is the first study comparing probiotic intervention with not only placebo treatment, but also metformin. This study has been approved by the ethics committee of the Medical University of Graz (EC number 32–230 ex 19/20).

Registration EudraCT number: 2020-000228-20. Clinicaltrials.gov identifier: NCT04593459.

Protocol version Version 1.5 dated 29th November 2021.

Keywords PCOS, Probiotic, RCT, Metformin

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Review > Expert Rev Clin Pharmacol. 2021 Sep;14(9):1081-1089.
doi: 10.1080/17512433.2021.1933433. Epub 2021 May 27.

Update on the effects of GLP-1 receptor agonists for the treatment of polycystic ovary syndrome

Maka Siamashvili [1], Stephen N Davis [1]

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Curr Pharm Des. 2016;22(36):5572-5587.

Gut-Brain Axis and Metabolism in Polycystic Ovary Syndrome.

Saydam BO, Yildiz BO[1].

Abstract

Polycystic ovary syndrome (PCOS) is a common and complex endocrine disorder, often accompanied and complicated by insulin resistance, glucose intolerance and obesity. Gut, brain and metabolism are highly related with each other in obesity and diabetes as well as in PCOS. Central nervous system regulates food intake through complex interactions of homeostatic and hedonic systems while gastrointestinal system contributes to food intake and metabolism via orexigenic and anorexigenic gastrointestinal hormones. Ghrelin is the only circulating orexigenic hormone whereas anorexigenic peptides include glucagon like peptide-1 (GLP-1), gastric inhibitory peptide (GIP), peptide YY (PYY) and cholecystokinin (CCK). Compared to healthy women, patients with PCOS show decreased or unaltered fasting ghrelin levels, along with decreased or unaltered postprandial suppression of this hormone. GLP-1, PYY and CCK show unaltered or decreased levels both in fasting and postprandial states in PCOS whereas fasting levels of another gut hormone, GIP is either unaltered or increased. Dietary interventions associated with weight loss or short term oral contraceptive use in PCOS do not alter fasting or postprandial levels of these hormones. However use of metformin is associated with an increase in ghrelin, PYY, GLP-1 and GIP in women with PCOS. GLP-1 agonists and bariatric surgery, both having a significant impact on gut-brain axis, appear to be effective therapeutic options in obese women with PCOS. Finally, alterations in gut microbiota and possible interactions with gut-brain axis in PCOS is a topic of interest. Understanding the relationship between PCOS and homeostatic and hedonic systems, gastrointestinal hormones, and gut microbiota as well as potential effects of different therapeutic interventions on these systems will provide further understanding and novel treatment opportunities for this syndrome.

PMID: 27426125

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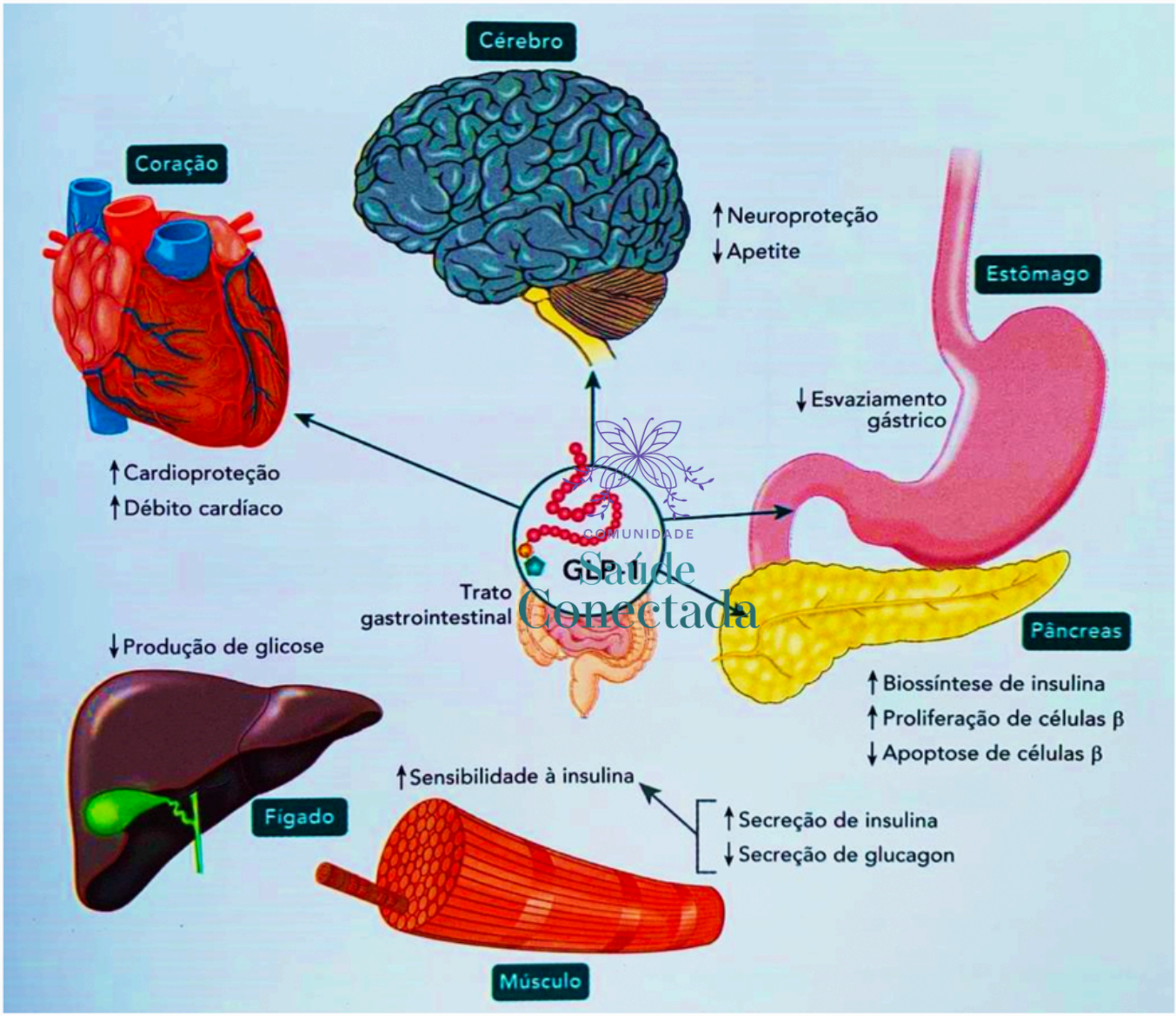

FONTE: PEREIRA, M; GOUVEIA, F. Modulação Intestinal – Fundamentos e Estratégias Práticas. 2019.

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> Adv Exp Med Biol. 2021:1328:513-522. doi: 10.1007/978-3-030-73234-9_36.

Boosting GLP-1 by Natural Products

Habib Yaribeygi [1], Tannaz Jamialahmadi [2 3], Seyed Adel Moallem [4 5], Amirhossein Sahebkar [6 7 8]

Affiliations + expand

PMID: 34981502 DOI: 10.1007/978-3-030-73234-9_36

COMUNIDADE Saúde Conectada

Abstract

The prevalence of diabetes mellitus is growing rapidly. Diabetes is the underlying cause of many metabolic and tissue dysfunctions, and, therefore, many therapeutic agents have been developed to regulate the glycemic profile. Glucagon-like peptide-1 (GLP-1) receptor agonists are a newly developed class of antidiabetic drugs that have potent hypoglycemic effects via several molecular pathways. In addition to synthetic GLP-1 receptor agonists, some evidence suggests that natural products may have modulatory effects on GLP-1 expression and secretion. In the current study, we conclude that certain herbal-based constituents, such as berberine, tea, curcumin, cinnamon, wheat, soybean, resveratrol, and gardenia, can exert an influence on GLP-1 release.

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DIETA (anti-inflamatória, antioxidante e de baixa carga glicêmica)
Low Carb
Plant Based
Dieta do Mediterrâneo
Dieta Cetogênica
Jejum Intermitente
Dieta low fodmaps
Investigar intolerâncias alimentares - Dieta de eliminação
Reposição de vitaminas
Exercício fisico
Cuidar da tireoide

PROTOCOLO DE DESINFLAMAÇÃO
CICLO DE ESTRATÉGIAS

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Exemplo 1

1 – 100 g de raiz + 2 ovos

2 – Carne 100 g + raiz 70 g + salada e legumes

3 – 1 iogurte 14 g + 20 g castanhas + 20 g chocolate 70%

4 – 1 lata de atum com especiarias

5 – Carne 100 g + arroz 2 col sopa + feijão 1 concha + salada verde

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Exemplo 2

Café
Pão artesanal + 1 fio de azeite
Queijo curado + orégano e majericão + tomate cereja
Fruta
Lanche
Castanhas + sementes e chá
Almoço
Peixe no azeite + ervas
Batata cozida + alecrim
Salada
Lanche
Whey Protein + fruta
Jantar
Sopa de legumes + frango
Caldo de ossos
Vinho (finais de semana)

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- Magnésio 2 x ao dia
- Acido alfa lipoico: entre 300 - 600 mg/ dia (não passar de 1000 mg/ dia)
- Mioinositol (mensageiro molecular para insulina): 2 a 4 g/ dia (doses fracionadas D-chiro inositol)
- Berberina: 350 a 500 mg, 2 a 3 x/ dia. Por 4 meses. Fazer intervalos e depois doses menores de até 300 g/dia.
- Vitamina D
- Ômega 3
- Sementes de linhaça: 1 a 2 col. de sopa/dia e semente de abóbora
- Peônia branca - Extrato seco (10:1): 0,5 a 1,5 g ao dia.

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- Chás e ervas: menta spicata (menta), alecrim, canela, chá verde, alcaçuz, amargos
- NAC: 1200 mg/ dia (doses fracionadas)
- Coenzima Q10
- Acil l carnitina (especialmente para melhorar a fertilidade) - 500 mg / dia
- Zinco: inibe 5-alfa-redutase e reduz queda de cabelo
- Melatonia: entre 0,5 a 3 mg/ dia
- Adaptógenos: ashwagandha, rhodiola, tulsi, reusai (inibe 5 alfa redutase), cogumelo Maitake
- Selênio
- Vitamina E, C
- Complexo B
- Picolinato de cromo

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Escolher de 2 a 3 suplementos, utilizarpor 3 a 4 meses e depois trocar por outros diferentes

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Chá de hortelã: é um chá digestivo e anti-inflamatório. Estudos identificaram que o consumo de 2 xícaras de chá de hortelã diariamente, durante um mês, promoveu efeitos antiandrogênicos, favorecendo a redução dos níveis de testosterona, em mulheres com SOP. Além disso, esse chá possui propriedades para auxiliar no tratamento de hirsurtismo (crescimento de pelos).

Chá de folha de amora: A folha do chá de amora é riquíssima em vitaminas e minerais essenciais. Além de possuir bastante cálcio, fortalece os cabelos, ajudando a diminuir a queda, além de regular os hormônios que estão em níveis anormais na SOP.

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Chá de Feno Grego: é uma planta medicinal que ajuda a regular os níveis hormonais e, por isso, pode ser usada para tratar vários tipos de problemas relacionados com o sistema genital da mulher. Além disso, também tem propriedades anti-inflamatórias. Ingredientes: 250 ml de água fria; 1 colher (de chá) de sementes de feno-grego. Modo de preparo: Juntar os ingredientes em um recipiente e deixar repousar por, pelo menos, 3 horas. Depois virar em uma panela e deixar ferver por 5 a 10 minutos. Por fim, deve-se coar a mistura e deixar amornar. Este chá pode ser tomado até 3 vezes por dia.

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Chá de unha de gato: além de ser uma planta medicinal com ação anti-inflamatória, também estimula a ovulação. Ingredientes: 1 colher (de sopa) de unha de gato; 500 ml de água. Modo de preparo: Colocar os ingredientes numa panela e deixar ferver. Depois de fervido, tampar e deixar repousar durante 10 minutos. Coar e beber o chá de tarde.

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Chá de uxi amarelo: propriedades anti-inflamatórias, alivia os sintomas da SOP e estimulando a ovulação. Ingredientes: 1 colher (de sopa) de uxi amarelo; 500 ml de água. Modo de preparo: Colocar o uxi amarelo e a água numa panela e deixar ferver. Depois de fervido, tampar e deixar repousar durante 10 minutos. Coar e beber o chá de manhã.

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Os **ovários** estão diretamente ligados à nossa **criatividade**, inspiração e leveza através dos nosso dons e talentos

As mulheres que possuem a Síndrome do Ovário Policístico (SOP) esquecem de viver seus sonhos. Na verdade, sequer lembram de inventar um sonho para si. Afinal, podemos dizer que os ovários são nossa fábrica de fertilidade. E isso não tem a ver apenas com gerar um bebê, mas também com nossa capacidade de criar, planejar e sonhar.

Mapa dos sonhos

Quando você não se permite ser fértil na vida ou viver momentos de prazer e diversão, os cistos aparecem no lugar de projetos realizados, orgasmos sentidos e amores vividos.

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Ovários Policísticos também sinalizam excesso de características **masculinas**

Os ovários são a parte mais masculina (Yang) e ativa do aparelho reprodutor feminino. Basta pensar que quando o ovário está exercendo sua real função, que é **liberar** o óvulo para ser fecundado, a mulher está na sua fase fértil.

Em tese, é neste período que ela está mais propensa a sair de seu mundo interno, ficando mais ativa, dinâmica, cativante, espontânea, sedutora, sexual, racional.

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Na SOP, o ovário aumenta de tamanho. E a explicação para isso é que esta mulher também está com sua questão mais ativa e masculina aumentada.

Ou seja, é alguém que está usufruindo menos de sua intuição, ou tendo dificuldade de acessar suas emoções ou mesmo seu lado maternal. Mulheres com ovários policísticos vivem num mundo racional, ou de muito trabalho.

Possuem um senso crítico forte, consigo e com os outros, além de se envolverem em tantos afazeres e tarefas, que sequer têm tempo de olhar para dentro.

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Pelo fato de **reprimir** suas emoções e não ter coragem de encará-las de frente e resolvê-las, essa mulher pode se comportar de forma mais dramática, briguenta ou agressiva. Por mais que seja elogiada, não consegue admitir que realmente é boa em algo, mantendo-se em seus julgamentos – seja consigo mesma ou com todos ao redor. Merecimento

Não é via de regra, mas algumas mulheres com Ovário Policístico, por terem um lado Yang mais evidente, podem ser mais ativas, dinâmicas ou ativas na cama. Mas, justamente por isso, acabam atraindo pares que possuem grandes características femininas, se relacionando, por exemplo, com pessoas mais intuitivas, espiritualizadas, sensíveis, introspectivas, emotivas. No entanto, outros podem ser os típicos “filhinhos de mamãe” ou até mais acomodados e preguiçosos.

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Ovários Policísticos e problemas com o pai

Por fim, também é comum que mulheres com ovários policísticos tenham tido problemas com a figura do pai – que pode ter sido alguém autoritário, ausente ou fraco. Assim, alimentam raivas enraizadas desta pessoa.

Como forma de compensar esse enfraquecimento do masculino, essas mulheres reproduzem este homem que está em falta em sua vida, criando-o dentro de si mesmas.

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Por isso, a maioria tende a ser o “homem da casa” ou o “homem da relação”. Essa é uma forma inconsciente de tentarem “corrigir” o papel do pai em suas vidas, para diminuir sua importância.

Então, essas mulheres tornam-se mandonas, controladoras, atuantes e cheias de iniciativa. Elas é que fazem tudo e assumem a maior parte das tarefas. O problema é que esquecem de pedir ajuda.

Num extremo oposto, também pode acontecer desta mulher desaprovar a postura e os comportamentos da mãe. Com isso, passa toda sua vida tentando não reproduzir os mesmos erros maternos e acaba repelindo o feminino em si. Neste caso, é necessário liberar o perdão, para reassumir o controle e a responsabilidade em sua vida.

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Acupuncture improves the emotion domain and lipid profiles in women with polycystic ovarian syndrome: a secondary analysis of a randomized clinical trial

Hui Chang [1 2], Baichao Shi [1], Hang Ge [1], Chengdong Liu [3], Lirong Wang [1], Chengcheng Ma [1], Lifeng Liu [1], Wanyu Zhang [1], Duojia Zhang [2], Yong Wang [4], Chi Chiu Wang [5], Xiaoke Wu [2 6]

Affiliations + expand

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Randomized Controlled Trial > J Pediatr Adolesc Gynecol. 2022 Jun;35(3):305-313.
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Outcomes of a Mindfulness-Based Healthy Lifestyle Intervention for Adolescents and Young Adults with Polycystic Ovary Syndrome

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JMIR Res Protoc. 2020 Jan 28;9(1):e14542. doi: 10.2196/14542.

Assessing Whether Meditation Improves Quality of Life for Adolescent Girls With Polycystic Ovary Syndrome: Protocol for a Randomized Controlled Trial

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